

End of Year MCQ & SCT Revision

Contents

MCQ Questions

2024 Exam - Page 2

2023 Exam - Page 8

2022 Exam - Page 17

2021 Exam - Page 21

SCT Questions

2024 Exam - Page 27

2023 Exam - Page 27

2022 Exam - Page 29

2021 Exam - Page 35

MCQ Questions

Post-op rhonci after CS c GA - How to prevent

Regional Anaesthesia

General anaesthesia increases aspiration risk in pregnancy. Regional avoids airway instrumentation and aspiration pneumonitis.

PID - Empiric Treatment

Doxycycline

To cover for Chlamydia - the most likely cause.

According to STI Guidelines the following should be used:

- Outpatient: Ceftriaxone + Metronidazole + Doxycycline
- Inpatient: Ceftriaxone + Azithromycin + Metronidazole

Cancer Asc. c Barrett's Oesophagus

Barrett's metaplasia → dysplasia → oesophageal adenocarcinoma

Most Common Form of Thyroid Cancer

1. Papillary - 80%
2. Follicular - 10%
3. Parafollicular - 10%
4. Anaplastic - < 1%

Hyperbilirubinaemia + Seizure + Pallor + Lethargy in a kid

Kernicterus

Severe unconjugated bilirubin crosses BBB → bilirubin encephalopathy → seizures/hypotonia.

Dramatic + Labile When Receiving Negative Feedback + Promiscuous - What personality disorder?

Histrionic

Linear Line of Vesicles on Lateral Forearm - What dermal pattern does this take?

Dorsal root ganglion

Zoster remains dormant in the DRG until reactivation along the areas it supplies.

Membranous Exudate on Tonsils + Organomegaly + Lymphadenopathy - What Ix sign is classic?

Peripheral smear with atypical lymphocytes

Classic for EBV infectious mono.

Paed c Split S2 systolic murmur @ 2nd L ICS

Atrial Septal Defect

ASD → constant RV volume overload → fixed wide S2 split

Monitoring Ix Required for Methotrexate Tx?

Monthly FBC

MTX → bone marrow suppression; FBC monthly is required

How to Speedrun to Megaloblastic Anaemia

Methotrexate

MTX is a folate antagonist - you need folate to make DNA.

RLQ Pain c Sever Metabolic Acidosis

DKA

DKA can cause irritation to the peritoneum, mimicking appendicitis.

BPH Mx

α blocker

Ix When Pt Comes in c BPH Sx

PSA

Need to rule out prostate cancer!

Courvoisier's Sign + Hx of Ulcerative Colitis in 11M

Choleangiocarcinoma

Painless + Palpable Gallbladder is always malignancy until proven otherwise (Choleangiocarcinoma or Pancreatic Cancer). UC increases your risk of developing Primary Sclerosing Cholangitis, which has a high risk of becoming Choleangiocarcinoma.

80F Urinary Urgency + Delirium

Oral Abx

Uncomplicated UTI treated c Oral Abx unless septic.

✓

Multiple Suicide Attempts + Emotionally Unstable

Borderline Personality Disorder

Self harm + Impulsivity + Unstable Relationships = BPD.

✓

Which Congenital Infection Causes Foetal Hydrops?

Parvovirus B19

Parvovirus → aplastic crisis → fetal anaemia → hydrops

?

Pelvic Pain + Fever 5/7 Post-Hysterectomy

Pelvic Abscess unless proven otherwise.

?

Fever Immediately Post Oesophagectomy Cause

Atelectasis

Causes direct low grade fever through inflammatory cytokines. Less than 24 hours post surgery is far too early for pneumonia being the cause.

✓

Mx of Mild Rash/Allergy with no signs Asc. c Anaphylaxis

Oral Antihistamine

✓

Floppy Baby + Tongue Fasciculations Dx & Area Impacted

Spinal Muscular Atrophy - Anterior Horn Cells

✓

5/7 Fever That Breaks Then New Onset Rash

Roseola infantum

Classic clinical findings of HHV-6

X

How do you confirm Psoriasis?

Bx of a psoriatic plaque

Histology shows acanthosis, parakeratosis, Munro microabscesses

✓

What Microbiology Do Candida Have?

Pseudohyphae

✓

Best Imaging for Suspected ACL Rupture

MRI

Flexed Knee + Joint Line Tenderness + Joint Locking

Meniscus Tear

Unilateral Erythematous/Swollen Hand c Past Hx of Similar Sx on Big Toe

Gout

Pulses Absent Distal to Knee, Which Artery Occluded?

Popliteal

Fibromyalgia Mx

CBT + Amitriptyline + TCA

Febrile Mastitis Mx

Oral Flucloxacillin

You would consider conservative Mx for the first 24 hours if no fever/systemic Sx.

BMJ Best Practice:

Antibiotics are indicated for patients with acute pain, severe symptoms, or symptoms lasting more than 12-24 hours; fever or any other signs of systemic infection; or positive microbiology studies.

As *Staphylococcus aureus* is the most common pathogen, antibiotics with activity against staphylococci should be used. If MRSA can be excluded by culture, or if MRSA is not prevalent locally, 10-14 days of oral dicloxacillin, cloxacillin, or flucloxacillin (depending on availability) is the initial choice. Cefalexin, a first-generation cephalosporin, may also be considered, but it has a broad spectrum of coverage and is more likely to promote the development of MRSA.

Pregnant Woman Exposed to Shingles - What Now?!

ZIg if she's seronegative to VZV.

Claudication + Chest Pain + T Wave Inversion

Unstable Angina

Rest pain + ECG Changes

Irreducible Inguinal Lump Mx

Urgent Surgical Referral

This is an incarcerated/strangulated hernia. Could become a surgical emergency.

Mx if Cervical Screening Test c CIN2 (High Grade) Results

Colposcopy + Biopsy

Wide Gait + Ocular Disturbances on Hx of EtOH Misuse Mx

IV Thiamine (and then glucose)

Lady Complains About Cost of Groceries. Dx?

Reasonable - not psych.

How to Test for Cat Allergy

Skin-prick and/or serum IgE

Pancytopenia Dx

Aplastic Anaemia

Impacts RBCs, WBCs, Platelets - Thus 'pan'cytopenia.

Worsening Migraines for Last 6/12 c Aura + Nausea and Vomiting

Do an MRI

Progressively getting worse + Nausea/Vomiting are Red Flags.

Pulmonary Oedema c HTN & LVH

IV Diuretics

Acute pulmonary oedema → IV furosemide first-line

How to Ix Obstructive Sleep Apnoea

Polysomnography

Man Thinks He's Being Hunted by the Terminator. What Type of Delusion is this?

Persecutory delusion

Woman Stopped Taking Her Meds for Mania... What is it probably?

Lithium

Two Years of Low Mood and Anhedonia

MDD

Lady Has Chocolate Cysts. What is she @ Risk for?

Infertility

Endometriomas impair ovulation and tubal function.

Husband Has Normal Sperm, She has 45-90/28 Cycles and Not Falling Pregnant. Dx.

PCOS

Oligomenorrhoea = Anovulation → Infertility.

Oligomenorrhoea + Multiple Cystic Lesion on Ovaries on USS + R Iliac Fossa Mass. Dx?

Endometriomas

Pelvic pain + Cystic Ovarian Mass = Endometriosis

Pregnant + Acute RLQ Pain. But Baby is Fine on CTG.

Treat suspected Appendicitis in pregnancy urgently.

Woman Presents @ 31/40 c Fundal Height 27 cm. Fundal Height @ 27/40 = 27 cm. Dx?

IUGR - Foetal Growth Restriction

Mx of VBAC G2P1 c Spontaneous Labour. 4 cm Dilation, 1.5 cm Effaced. Nil Scar Tenderness. Happy Baby on CTG. BP 130/80. Nil Rupture of Membranes

Continue partogram c normal birth

This is a low risk VBAC c good progress, let her try deliver vaginally.

46F c Heavy Irregular Periods + Fatigue. What Ix Would You Get

TFTs (TSH)

16/12 c 10/7 Fever + Urinary Frequency. Now Not Feeding + Signs of Shock

Sepsis (From UTI obvslly)

16F c 5/12 Amenorrhoea + Thin + Mother Concerned

Anorexia nervosa

Low BMI + restrictive eating → hypothalamic amenorrhoea

10/12M c Bowing of Legs - What Would You See on Xray?

Metaphyseal cupping/flaying

This is the classical radiological sign of Rickets.

3F c Purulent Malodorous PV DC. Dx.

Foreign body in vagina

Foul, blood-stained or malodorous discharge in a prepubertal girl is classic for a retained foreign body (often toilet paper), rather than STI or vulvovaginitis.

Fever + Diarrhoea + Blood in Stool. Likely Organism?

Campylobacter

Campylobacter jejuni commonly causes inflammatory, bloody diarrhoea with fever; rotavirus typically causes watery, non-bloody diarrhoea.

How Long Are You Immunocompromised Post-Splenectomy?

> 10 Years

Asplenic patients remain at lifelong risk of overwhelming sepsis from encapsulated organisms, though highest in first few years.

3M c Maroon Coloured Stool + Hypotension. Likely Cause?

Meckel's Diverticulum

Painless, sometimes massive lower GI bleeding in young children is classic for Meckel's, not UC or diverticular disease.

Mx of Epiglottitis c Drooling

Systemic steroids (plus airway + IV antibiotics in real life)

Epiglottitis is an airway emergency requiring airway management and IV antibiotics; steroids are used as adjuncts, while nebulised adrenaline is more for croup.

What is Coffee Bean Sign on AXR?

Sigmoid Volvulus

Mother Has Low Folate. What is Baby @ Risk of?

Neural Tube Defects

Heart Murmur + Back Pain... What Rheum Ix Do You Need?

HLA-B27

Aortic regurgitation with inflammatory back pain suggests ankylosing spondylitis, which is strongly associated with HLA-B27.

Split S2 Systolic Murmur @ L 2nd ICS. Dx.

Pulmonary Valve Stenosis

An ejection systolic murmur at the left upper sternal edge is most consistent with pulmonary stenosis; aortic stenosis is best at the right upper sternal border and radiates to the neck.

Slapped Cheek Rash that Spares Nasolabial Folds

Erythema infectiosum - Parvovirus B19

Bright "slapped cheek" rash with reticular body rash and nasolabial sparing is classic for fifth disease.

Vaccines to Give Pregnant Women

Flu + dPta (to protect against Pertussis)

Which Nerve is Involved in Crocodile Tears Syndrome?

CN VII - Facial Nerve

Misrouting of facial nerve parasympathetic fibres to the lacrimal gland causes tearing while eating.

24F Seen 4x Drs Convinced She Has HIV, But No Sx & -ve HIV Ab Tests. What Dx?

Illness Anxiety Disorder

Persistent preoccupation with having a serious illness despite medical reassurance points to illness anxiety, not GAD.

When Do You Test For PCOS?

3/28 (Early Follicular Phase)

Gonadotrophin levels are interpreted in the early follicular phase (days 2–5); day 3 is standard.

Twins. One Has Oligohydramnios, The Other Polyhydramnios... What's Going On?

Twin-to-Twin Transfusion Syndrome

In monochorionic twins, unbalanced placental shunting leads to donor (oligohydramnios) and recipient (polyhydramnios).

Metabolic Acidosis + Fever cout DKA Sx. Dx?

Sepsis

Fever plus metabolic acidosis without typical DKA features suggests lactic acidosis from sepsis.

8/52 Amenorrhoea + Urinary Frequency + 2 Past Ectopics

Ectopic Pregnancy - Until Proven Otherwise

Hearing Loss + Facial Droop + Facial Weakness - Where's the Lesion?

Pontine Angle - Cerebellopontine Angle

A lesion at the CPA (e.g., acoustic neuroma) affects CN VII and VIII → facial weakness + hearing loss.

How Do You Adjust Treatment of Addison's if they get secondary illness?

Increase hydrocortisone and leave fludrocortisone unchanged

In stress/illness you stress-dose glucocorticoids, while mineralocorticoid dose usually stays the same.

Drugs That Cause Myopathy

1. Statins - Direct effect
2. Thiazides - Indapamide can cause electrolyte disturbances.

What Type of Antihypertensives Can Cause Gout?

Thiazides

Thiazides reduce urate excretion and commonly precipitate gout attacks.

Intermittent Colicky Abdo Pain in Netball Captain. What Might She Have?

Functional Abdominal Pain

Normal growth with recurrent abdominal pain and no red flags in a school-aged child suggests functional pain.

Farmer c Social Withdrawal, High EtOH Intake, Low Motivation and not Eating. What Mx?

1. CBT
2. SSRI/SNRI
3. TCA

Which Bipolar Medication is Safest During Pregnancy?

Lamotrigine

Lithium, valproate and carbamazepine are teratogenic, and antipsychotics carry more metabolic risks.

30F Back Pain + Anxious About People Getting Sacked @ Work Because of Back Injury. What makes her high risk for developing a chronic pain disorder?

Anxiety around work

BO > 5/7 Ago. Nil Flatus 2/7. How Would You Ix?

CT abdomen c Contrast

CT with contrast is the best test to confirm obstruction and identify level/cause in an adult.

Pleuritic Chest Pain + ST Elevation Across Many Leads. Dx?

Acute Pericarditis

Diffuse ST elevation with pleuritic pain and low-grade fever is classic for acute pericarditis.

50M PMHx MI + Palpitation + Tachycardia + Recent 15/24 Flight + ECG Suggestive of AF. What Ix next?

Echo

33F Public Health Contact to Say She Has Been Contact c Chlamydia. What is the best way to reduce spread?

Contact her partners for testing/treatment

Pt asks for Sleeping Pills. Reports Insomnia + Low Mood @ Beginning of Week + Drug Use of Weekends. Why Fatigue?

Stimulant Substance Abuse

Bleeding Gums + Rash + Prolonged PT & aPTT

Deficiency of Vit K dependent Factors II, VII, IX, X

Prolonged PT and aPTT suggests multiple coag deficiencies - suggestive of Vit K Deficiency or Liver Disease.

Breast Nodule Bx Contains Fat + Stromal + Epithelial Cells. Mx?

Local Excision

A lesion with mixed fat and glandular/stromal tissue is likely a benign hamartoma or fibroadenolipoma, treated by lumpectomy.

25F has 1.8 cm Ovarian Cyst @ 12/28. What Mx?

Follow up USS in 3/52

This likely represents a dominant follicle; small simple cysts in premenopausal women are observed and re-imaged.

Post-Tibial Repair: 8/24 Severe Pain c Passive Dorsiflexion. Mx?

Calf Fasciotomy

Pain out of proportion and on passive stretch = compartment syndrome → urgent fasciotomy.

65M Hyperinflated Chest + SOB + 40PY Smoking Hx, What is the next step in the Mx of his ?COPD

Spirometry

COPD is confirmed with Obstructive pattern on Spirometry.

Which Drug Class Can Mask Hypoglycaemia?

β Blockers

Girl Presents c Extreme Phobia of Dogs. Wants to Improve as BF has German Shepherd. Mx?

Graded exposure - start with images

Fundoscopy Results: Cotton Wool Spots, Microaneurysms, AV Nipping, Hard Exudates, Dot/Blot Haemorrhages. Dx?

T2DM

These are classic signs of diabetic retinopathy.

How would you monitor a 4.5 cm AAA?

Duplex USS

For AAAs 3–5.4 cm, serial ultrasound is recommended to monitor growth.

4/12M Parents Think Swollen Scrotum. Nil Lump But One Spermatic Cord Longer Than Other. Mx?

Reassure parents nil pathological lump

Asymmetry of palpable mass is usually benign.

Erythema of Knee in HF Pt Post-Frusemide Tx. Suspected Joint Aspirate Results?

Crystal Arthritis - Uric Acid/Gout

SOB Post 20/24 Flight. What Ix?

CTPA - Rule out PE

Jaw Claudication + Other Temporal Arteritis Sx. Initial Mx?

High Dose Prednisolone

Suspected giant cell arteritis is treated immediately with high-dose steroids to prevent vision loss.

Dyspnoea When Walking 100 m to Mailbox but Nil @ Rest. What is their NYHA?

Class III

Class	Description	Symptoms
I	No limitation of physical activity	Ordinary physical activity does not cause undue fatigue, dyspnoea, palpitations, or angina
II	Slight limitation of physical activity	Comfortable at rest; ordinary activity (e.g. walking several blocks, climbing stairs quickly) causes symptoms
III	Marked limitation of physical activity	Comfortable at rest; less-than-ordinary activity (e.g. walking <100–200 m, climbing 1 flight of stairs slowly) causes symptoms
IV	Unable to carry out any physical activity without symptoms	Symptoms at rest; discomfort increases with any physical activity

44F ~~5/12~~ Amenorrhoea & Previous Heavy Periods. First Ix?

β -hCG

Abdominal Mass from Xiphisternum → Umbilicus, But Only When Sitting Up. Not c Cough or Strain. Cause?

Divarication of rectus abdominis

Separation of the rectus muscles presents as a midline bulge on sit-up, not with cough/strain like a hernia.

Most Appropriate HRT for 56F c Uterus

Continuous Oestrogen + Continuous Progesterone

Anyone with an endometrium needs to have progesterone to ensure no unopposed oestrogen causing cancer risks.

Chest Pain c Arm Radiation + Missing L Radial Pulse. Dx?

Aortic Dissection

Sudden chest pain with pulse asymmetry is classic for dissection.

31F c Chest Pain, Headache, Neuro symptoms, Diarrhoea. But Normal Bloods Whenever tested. Sx started 3 years ago due to missing shifts for sick leave.

Somatic Symptom Disorder

Persistent physical symptoms with frequent healthcare use and no organic findings is most consistent with somatic symptom disorder. Factitious unlikely as she willingly lost her job over this.

10M, HB 90, WBC 200000, Platelets 15000 + Mediastinal Mass

Acute Lymphoblastic Leukaemia

Patient Comes c Weakness. What Sx would make you think MS?

Optic Neuritis

Painful monocular visual loss (optic neuritis) is a classic presenting feature of multiple sclerosis.

Man c Difficulty Speaking, it takes time for him to get words out but you understand what he's trying to say. What is this?

Motor Aphasia

Non-fluent, effortful speech with relatively preserved comprehension suggests Broca's aphasia.

Epigastric Pain + Weightloss + Vomiting + Succussion Splash. Dx

Chronic Duodenal Ulcer

Succussion Splash indicates Gastric Outlet Obstruction. The most common cause of this is a chronic duodenal ulcer.

Pleuritic Chest Pain. Nil SOB. Recurs every 5-10/60 & is associated with nocturnal cough

GORD

COPD would have SOB.

23F Mild Hypercalcaemia + Normal ALP/EUC/FBC

Primary Hyperparathyroidism

Asymptomatic, mild, Hypercalcaemia is mostly likely primary hyperparathyroidism.

Immediate Mx of Septic Knee

Joint Aspiration

Pt Asks About Screening for Prostate Cancer Risk. What Should You Do?

Discuss pros and cons of testing c patient

PSA/DRE screening is controversial in their specificity. Shared decision making with patient so they know the benefits and risks is the best approach.

11M Extreme Testicular Pain & Won't Tolerate Examination. Mx?

Immediate surgical exploration

This is testicular torsion until proven otherwise. Time is tissue. **Do not wait for an USS.**

Claudication c 90% Stenosis of Femoral Artery. Mx?

Angioplasty balloon and stent

Short-segment, high-grade stenosis in a large artery is best managed by endovascular angioplasty $\pm$ stenting.

Straw Coloured PV DC 3/52 post Antepartum Haemorrhage c Recent Sex. What is the fluid?

Liquor - Rupture of Membranes

Straw coloured fluid post APH is likely Liquor not Semen.

Aortic Regurgitation c EF < 40% + Heart Failure Sx. Mx?

Aortic Valve Replacement

6/24 Post Thyroidectomy for Toxic Multinodular Goitre. Woman now aggressive and irritable. Why?

Thyrotoxic crisis

Surgery on the thyroid can cause thyroid storm.

40F Fijian NSEB Present c SOB. Xray Shows Right Middle Lobe Pneumonia. Treatment?

IV Abx

4M c Oedema Around Eyes and Legs; Not Sick Tho. Mx?

Prednisolone

Most childhood Nephrotic Syndrome is Minimal Change Disease - Treat c High-dose oral steroids.

3M Wets the Bed. Should parents be upset?

Reassure parents it is developmentally normal until 5 years.

Parkinson's Patient Presents c Hallucinations. Cause?

Medication Side Effect - Dopamine does this.

Left Pleural Effusion + Crackles on Auscultation What Ix next?

Echo

Dyspnoea on Exertion + Q Waves on ECG. What Ix should you get?

BNP - To see the extent of the HF she has from her past MI.

Child c Petechiae and Bruising. What is the first Ix you should get?

FBC

Test to see platelets and look for leukaemia. If normal consider coags.

Radicular Back Pain c Nil Neuro Sx. In Addition to Analgesia, What is the Mx?

Encourage mobilisation

4/12F Bronchiolitis + Normal Sats + Mild Subcostal Retractions. Mx?

Supportive Care

Mild bronchiolitis is managed with hydration, nasal suction, and observation; bronchodilators are not routinely indicated.

36F c Dyspareunia After Total Hysterectomy + Salpingo-oophorectomy. Mx?

Oestrogen Only HRT

Without a uterus you don't need progesterone to protect you from oestrogen causing endometrial cancer.

Epididymitis Sx c Thickened Vas Deferens. What Ix?

USS

Scrotal/inguinal ultrasound is the first-line imaging to characterise vas deferens thickening.

Down's Baby c Soft Non-Tender ^{fistulised} Abdomen

Duodenal Atresia

Down syndrome is strongly associated with duodenal atresia causing bilious vomiting and "double-bubble" sign on imaging.

9/12F c 10/60 of Head Bobbing + Trunk Flexion + Extension of the Arms. What's Going On?

Infantile Spasms (West Syndrome)

CXR c Pleural Effusion in Middle Zone. What Should Be Done About the Fluid?

Lateral Chest ^{Drain} Drain/Aspiration

Loculated or mid-zone effusions are best drained from the site where the fluid actually collects, usually laterally.

73F Presents c 2/52 Recurrent Falls + Postural Hypotension + DM + Febrile + MMSE 27. Next step in her Mx?

Urinalysis (She has delirium from a UTI)

21F Escitalopram + Mirtazapine is Agitated + Fever + Clonus. Dx?

Serotonin Syndrome

Dactylitis/Sausage Fingers are Asc. c which form of Arthritis?

Psoriatic Arthritis

What are the Features of Neuroleptic Malignant Syndrome?

Raised CK c hyperthermia and 'lead-pipe' rigidity

30M From Cyprus c Microcytic Hypochromic Anaemia. Likely cause?

β Thalassaemia Trait

Most Common Cause of Croup

Parainfluenza Virus

What Joints Does OA Impact?

Weight-bearing joints

What Joints Does RA Impact?

Symmetrical small joints - Classically MCP & PIP joints

Returned from Africa c Diarrhoea - Likely organism?

E. coli

Enterotoxigenic E. coli is the most common cause of traveller's diarrhoea.

What Defect Leads to Hypoxia Even if Baby Breathing 100% O₂?

Transposition of the Great Vessels

Pulmonary and Systemic run in parallel. Where Pulmonary circuit will have 100% SpO₂ and the Systemic will have 0% SpO₂.

Post R Hemicolectomy Not Eating - Why Is Urea Falling

Reduced protein catabolism

With low protein intake (such as subsisting on IV dextrose) the body catabolizes less protein, so urea levels decrease.

Mx of a Hydrocele in 4/12M

Wait until 15-18/12.

Most infant hydroceles resolve by 2 years.

18/40, But Fundas @ Pubic Symphysis. What Now?

USS to confirm viability and growth.

Most Common Hallucination Type in Schizophrenia

Auditory - Third person command hallucinations are the most common.

First Line Mx of OA in Knee

Regular Paracetamol

Then add exercise and weightloss.

Resting Energy Intake of Adult Male

2000 kcal/day

F Infertility since Curettage + Painful, but not heavy, Periods. How to Ix?

Hysteroscopy

Likely Asherman's Syndrome (Intrauterine Adhesions)

Definition of Male Factor Infertility

Low number of quality sperm that is sufficient to fertilise an ovum.

Fine Tremor Since Being on Li for 6/12. First step to Ix.

Check Li Level

You must check for Li toxicity before treating the tremor.

Risk of Colorectal Cancer c Crohn's

10%/20 years c Crohn's

Child c Purpuric Rash from SE Asia. Dx? Mx?

- Meningococcal/gram -ve sepsis
- Start IV Abx ASAP

2/12 Epigastric Pain + Weightloss + Mild Dehydration + Anorexia. Most Concerning Dx?

Gastric Cancer

45F Concerns She Has Osteoporosis. PMHx Broken Hip in MVA. Grandma had Osteoporosis. What do you tell her?

Education on lifestyle, encourage Ca/Vit D supplementation. DEXA scan not warranted.

Biggest Risk Factor for Eczema

FHx Atopy

Atopic dermatitis strongly clusters in families with asthma, hay fever, eczema.

Wilson's Disease in Baby 1. What is the odds of Wilson's disease in next baby?

25% - Autosomal Recessive

What Molecule is ^{Apl. c} Steatosis in Cells?

Triglycerides

11/12F c Wheeze, Vitals NAD. Mx?

Supportive care and rv in 24/24.

First line Tx for Stress Incontinence

Pelvic Floor Exercises

Ulcer on Foot. What Must Be Done Before Starting Compression Therapy?

Ankle-brachial Index

Compression Tx helps with venous ulcers. But if you do it to an arterial ulcer, the leg already doesn't have enough blood flow. Bad news bears.

Most Important Side Effect of Clozapine

Agranulocytosis

2/12 Child c Flu Sx. Smoker Dad Has a New Onset Cough for 2/12. What Abx Should be Given to Both?

Azithromycin/Clarithromycin - A Macrolide

This question is difficult. It relies on the knowledge that Pertussis should be ruled out for all URTIs in children < 3/12 and that all new onset infectious coughs > 2/12 in duration for adults should have a DDx considering pertussis. It also relies on you knowing that macrolides are the mainstay of treatment for pertussis.

28F c Green Nipple DC. How to Ix?

Breast USS - to see if there's Duct Ectasia

Imaging for Choledocolithiasis

- MRCP - If your hospital has it
- ERCP - Dx and therapeutic

Safest Epileptic For Pregnant Woman's Foetus

Lamotrigine

RUQ Pain + Fever + Jaundice (Charcot's Triad)

Ascending Cholangitis

Antihyperglycaemic Most Likely to Cause Hypoglycaemia

Gliclazide

Sulfonylureas stimulate insulin release causing hypos.

70M Back Pain + High ALP + Paget-like Xrays

Paget's Disease

Paget's is associated c High ALP with thickened deformed bone on imaging.

Baby has Upturned Small Nose + Low Birth Weight + Microcephaly. What Toxin Did Mum Take During Pregnancy?

EtOH

These are the signs of Foetal Alcohol Syndrome.

30F 30/40 Uses Cocaine. Reduced Foetal Movements + Abdo Pain + Fundal Height 32 cm.

Placental Abruption - Silent Abruption

5 Years Survival Rate of Stage II Oesophageal Cancer

30%

Child c Mucosal + Retinal Bleeding and Low Platelets. Dx?

Immune Thrombocytopenia

Isolated thrombocytopenia with bleeding in an otherwise well child suggests ITP.

90M c Parkinsons. SBO Sx. How to Ix?

AXR

Progressive Weakness of all Facial Muscles on one side

LMN lesion of **facial nerve** at stylomastoid foramen.

LMN facial palsy affects the whole ipsilateral face (including forehead).

Community Acquired Pneumonia Mx in Paeds

Augmentin - Amoxicillin + Clavulanate

Woman has pink tissue in her cystic tissues. What does she have?

Endometriosis

Swollen, Red, Painful Big Toe

Acute monoarthritis of first MTP with erythema is classic podagra.

1/60 Post-Partum. What Can You Give to Help Deliver Placenta?

10 IU Oxytocin IM

Source: <https://litfl.com/procedure-postpartum-haemorrhage/>

28/40 Headache + RUQ Pain + High BP. Likely Dx?

Pre-eclampsia

Homeless Man c EtOH Use Disorder + Diarrhoea + Chest Infections + Epigastric Pain

Pancreatitis

Alcohol-related chronic pancreatitis → malabsorption diarrhoea and epigastric/back pain.

Patient Receives Ethambutol for TB Mx. What Do You Need to Monitor?

Visual Fields/Visual Acuity

Ethambutol can cause optic neuropathy (colour vision and visual field defects), so vision monitoring is key.

MOA Naltrexone

Opioid Receptor Antagonist

Naltrexone blocks μ -opioid receptors to reduce the rewarding effects of alcohol/opioids.

↑
N

PhD Student Scared of Flying. Has Conference in Jamaica in 3/12. What Do?

CBT - c exposure based therapy

Chronic Productive Cough + Clubbing. Dx?

Bronchiectasis

Chronic wet cough with digital clubbing is classic for bronchiectasis rather than chronic bronchitis or emphysema.

Best Test To Understand Irregular Cycles for Fertility Assessment

Mid-luteal Progesterone

Day-21 (mid-luteal) progesterone assesses whether ovulation is occurring; irregular cycles usually mean anovulation.

Three Babies c Jaundice

Baby 1: Pale Stools - Conjugated Jaundice c Acholic Stools

Biliary Atresia

Baby 2: 3/52 & Thriving. Drinking HEAPS of Breast Milk. Now Yellow? D:

Breast Milk Jaundice (This is benign)

Baby 3: Progressive Jaundice @ 5/52. How would you investigate?

LFTs + Abdo USS

Hx Suggestive of Pancreatic Cancer. Would she have Conjugated or Nonconjugated Hyperbilirubinaemia?

Conjugated

Pancreatic head tumors obstruct the common bile duct → conjugated jaundice.

26F Had IUD Inserted 2 Years Ago. Now Has Adnexal Mass. What Organism Caused This?

Actinomyces

Long-term IUD use is associated with Actinomyces infection causing pelvic masses/abscesses.

Woman c BPD in ED Keeps Telling JMO That “He’s the best doctor in the world”. What is this behaviour called?

Splitting

BPD defence mechanism – others are seen as all good or all bad, rapidly flipping between idealisation and devaluation.

Hx Consistent c MS. What Imaging Should You Order?

MRI Brain/Spine

Milky Vaginal DC. Likely Organism?

Candida albicans

3F Has Sore Knee. Cannot Weight-bear. Bloods Show Profound Cytopenia. What is next step?

Bone Marrow Aspirate

Bone pain plus cytopenias suggests leukaemia; marrow exam is key to diagnosis.

Woman c Lesion on Vulva + Dysuria that has progressed to retention

Herpes

4M Cough c Widespread Coarse Crackles

Bronchopneumonia

Diffuse coarse crackles with cough and systemic signs fits broncho-pneumonia.

3M c Recurrent Otitis Media + Aggression + Limited Vocab. Mx?

Audiology referral - Hearing loss mimics autism

20M Mucousy Diarrhoea + Pain Resolved c Opening Bowels. Colonoscopy showed nil pathology. Dx.

Irritable Bowel Syndrome

70F Bipolar, Brought in by Daughter, c 1/52 Confusion + Extreme Thirst + Abdo Pain + Coarse Tremor + Nausea Vomiting. What drug is the cause?

Li

Thyroid Tumour Comes Back +ve for Calcitonin. What Sort of Cancer is it?

Medullary Thyroid Carcinoma

MTC arises from parafollicular C cells, which secrete calcitonin.

What Differentiates Delirium from Schizophrenia?

Attention is preserved in Schizophrenia

What Differentiates Delirium from Dementia?

Delirium fluctuates

10F Thrown From Horse. Unconscious 5/60 + Broken Arm + Bruising over Abdomen + Tachycardia + Hypotensive. Where might she be bleeding?

Intrabdominal

Postop Extubate → Becomes Unresponsive → HR + BP NAD, SpO₂ 73%. Placed on 100% O₂ Non-rebreather. ABG pH 7.3, Low CO₂ low PaO₂, Normal Bicarb. What now?

Reintubate

4/12M c Constant Vomiting. What Acid-Base Problem will they have?

Metabolic Alkalosis + Low K

78F Admitted For COPD. Now Acting Psychotic. What Caused This?

Steroids

Systemic corticosteroids can precipitate steroid-induced psychosis.

77F Regular Pt Visits 6/12 for HTN Mx. Neighbour thinks she's mad. She thinks the neighbour is shining lights into her house and wants to build a tennis court on her property. No issue remembering appointment times without reminders and always turns up on time. What's going on?

Late-onset Schizophrenia

Systematic persecutory delusions with preserved day-to-day function and no clear cognitive decline = late-onset schizophrenia.

10

~~14~~ **14M Hasn't Undergone Puberty and is Short "Bone age of a 9 year old". Dad is 75th Percentile for Height. What's going on?**

Constitutional delay of growth and puberty

This is rare but benign. He'll get to normal height when it kicks in. No red flags mentioned.

4M Viral exacerbation of asthma. On Preventer that Mx'd ok over Winter. What is the next step in Mx?

Short course of oral steroids for acute Sx

SCT Questions

20M expresses suicidal thoughts. You're considering involuntary admission

- SADPERSONS:
 - Male
 - Age
 - Depression
 - No Spouse
 - Ongoing Attempt

Then you learn he has a gun and lives alone

- SADPERSON:
 - No supports
- Access to lethal means without a method of preventing him of using it.

Confidence in decision

Very high - High acute suicide risk, he cannot safely DC.

20M expresses suicidal thoughts. You're considering DC c Psych FU

- SADPERSONS:
 - Male
 - Age
 - Depression
 - No Spouse
 - Ongoing Attempt

Then you learn mother can supervise him

- Safe environment c reliable supervision reduces immediate suicide risk.

Confidence in decision

Higher - assuming she is willing to take him

6M c Henoch-Schoenlein Purpura. You're considering AXR

- HSP can cause intussusception and bowel infarct

Then you learn he has a soft non-tender abdomen

- Low suspicion for GIT pathology

Do you still want AXR

Less likely to be indicated

6M c Henoch-Schoenlein Purpura. You're considering Abdominal USS

- See above

Then you learn that the belly is distended

- Distension suggests obstruction.

Do you still want USS

More indicated. USS is the best diagnostic tool for intussusception.

3M had a reaction post-vaccination. You're thinking Anaphylaxis.

- Initial DDx:
 - Anaphylaxis
 - Vasovagal
 - Febrile Convulsion

Then you learnt that the reaction happened c. 2/60 of injection

- Timing fits

Confidence in Dx

Much more likely

3M had a reaction post-vaccination. You're thinking Vasovagal.

- See above

Then you learn HR was 120

- Vasovagal response causes bradycardia and hypotension

Confidence in Dx

Much less likely

30M presents c spondyloarthropathy. You're considering ordering HLA-B27 testing.

- +ve test confirms ankylosing spondylitis

Then you learn he has a stiff and sore back

- Suggest axial involvement - characteristic of ankylosing spondylitis

Does this impact appropriateness of testing?

Testing becomes more appropriate - as other consistent Sx.

32F c RA-like Sx. You're considering Rx MTX.

- Methotrexate is the first-line treatment for confirmed RA.

Then you learn that her anti-CCP is > 100

- High anti-CCP has high specificity for RA and predicts more severe erosive disease.

How does this impact your decision

MTX Tx more appropriate to prevent long term severe Sx

CT Shows Retroperitoneal fluid. Is Leaky Aneurysm more or less likely?

More likely

CT Shows extraluminal gas +/- Bowel Wall Thickening +/- Abscess. Is Complicated Sigmoid Diverticulitis more or less likely?

More likely

CT shows hydronephrosis +/- Proximal ureter dilation +/- visible stone. Is Urolithiasis more or less likely?

More likely

40M Previous Renal Stone Comes to GP for RV. Is well now. You're considering sending home.

- Seems reasonable.

Then you learn he haematuria

- New haematuria may indicate:
 - New stone
 - Obstruction
 - Infection
 - Malignancy

How likely are you to DC now?

Low - I want to FU on the above DDx

Woman with sarcoid presents c red eye

- Red painful eyes are often conjunctivitis or nonspecific irritation
- However, someone with Sarcoid should have a high suspicion for anterior uveitis

Then you learn she recently had an eye injection

- This could precipitate sarcoid-related uveitis and thus require ophthal referral

How likely is this uveitis?

Very Likely

Elderly lady is confused. You're considering delirium

- First Dx of acute elderly confusion → delirium

But she is able to maintain attention with you

- Key feature of delirium is loss of attention.
- If attention intact consider:
 - Mood disorder
 - Psychosis
 - Dementia

Confidence in Dx

Much less likely

Elderly woman is confused. You're thinking delirium

- See above

Then you learn she's been seeing her dead dog for 2 years

- Chronic vivid hallucinations are extremely unlikely in an acute condition like delirium.
- Consider chronic psychotic disorder:
 - Schizophrenia
 - Psychotic depression
 - Lewy body dementia

Confidence in Dx

Unlikely

32F c /7 Hx mild cramping abdo pain + PV bleeding. Last normal period was 6/52 ago. You were considering menstrual pains

- I would consider Ectopic or miscarriage... But c'est la vie

Then you learn she has PCOS

- Associated c Oligomenorrhoea, with prolonged cycles and irregular bleeding.
- Makes this plausible. I would still be concerned about important DDx first.

Confidence in Dx

Low

50M you suspect Colon Cancer

- Clinical Features:
 - Change in bowel habits
 - Weight loss
 - Blood in stool

Then you learn his DRE is NAD

- DRE only assesses the distal rectum - doesn't give much of a picture further up:
 - In fact: Bleeding on DRE would be reassuring as suggestive of haemorrhoids.

Confidence in Dx

About the same

Baby comes in yellow. You're thinking Breast Milk Jaundice

- Breast-fed babies can get benign jaundice from bilirubin in breast milk.

Then you learn the baby is 12/24 old

- Breast Milk Jaundice takes 3-5/7 for onset.
- Jaundice within the first 24 hours of life is pathological until proven otherwise

Confidence in Dx

Extremely unlikely

Neonatal jaundice + anaemia. You're thinking alloimmune haemolytic disease of the newborn

- Rh incompatibility between mum and baby lyses baby's RBCs

Then you learn that mum is O⁺

- Mum can't have anti-D Ab... Because she has D.

Confidence in Dx

Extremely unlikely

16F with arthritis. You're thinking Psoriatic Arthritis

- I suppose it's in the DDx

Then you learn dad has psoriasis

- Psoriatic Arthritis has a strong Family Asc. c psoriasis.

Confidence in Dx

Much more likely

Man comes with respiratory Sx. You're considering getting a CXR

- To find out if anything wrong with lung/heart.

Then you learn he has haemoptysis

- Red flag for serious pathology:
 - Malignancy
 - TB
 - Pneumonia c necrosis

How does this impact your decision

Much more likely to want CXR

40F Jaundice + RUQ Pain + Hypotension. Considering ERCP

- To decompress bile ducts for severe ascending Cholangitis

Then you learn she has rigidity

- Suggests peritonism

How does this impact decision

May delay ERCP - May need to go with direct laparotomy/laparoscopy first.

40F c Jaundice + RUQ Pain + Hypotension. You're considering cholecystitis.

- This actually sounds more like severe cholangitis

Then you learn she doesn't have a fever

- Sepsis c/w fever is possible. So could still be cholangitis.
- Makes simple cholecystitis more likely.

Confidence in Dx

More likely - but not confirmed

8F c Asthma Sx. You're considering allergy testing

- Allergic asthma is a thing. Worth testing.

Then you learn she has a Hx of atopy

- Hx of eczema/allergic rhinitis/asthma all increase pretest probability of allergic triggers

Does this increase or decrease utility

More likely to be useful

25M has sore throat. You're thinking of Rx Abx

- Could be viral... Probably more likely.

Then you discover exudate on tonsils

- Exudate is within the feverPAIN score system. They may now qualify for Empiric Abx

Does this change your thinking?

More likely to Rx

PV bleeding. You're considering ectopic pregnancy

- High on DDx of any woman with abdo pain/bleeding

Then you learn -ve β hCG

- False -ve pregnancy tests are extremely unlikely.

Confidence in Dx

Much less likely

Patient has a positive Murphy's Sign. You're considering abdominal CT.

- Positive Murphy's makes acute cholecystitis likely. Imaging would be warranted.

Then you learn GGT + ALP NAD

- Normal cholestatic enzymes suggest no biliary obstruction.
- Doesn't comment on health of gallbladder though.

Does this change Mx?

No - Although USS would still be better.

18F has Dysuria + Frequency. You're thinking UTI

- Checks out.

Then learn the Urinalysis shows +ve for Leukocytes + -ve for Nitrites

- +ve Leukocytes is convincing for inflammation/infection.
- -ve Nitrites says no nitrite forming bacteria.

Confidence in Dx

Very slightly less

20F has Dysuria + Frequency. You're thinking Pyelonephritis.

- Sx to look out for:
 - Fever
 - Flank Pain

Then you learn both Leukocytes and Nitrites -ve.

- ?Sampling issue
- If true makes Pyelonephritis much less likely

Confidence in Dx

Much less likely

22F has Asthma. You want to Rx SABA

- First line Mx of asthma

She does not want to use a puffer as got 'shakey' last time

- Tremor is a known side effect of β agonists - may reduce adherence.
- SABA is clinically appropriate. Consider shared decision making.
- If absolutely does not want SABA consider low dose ICS as preventer, reducing her reliance on the SABA.

How much impact does this have on you?

A little bit.

Bipolar Pt Noncompliant c Sertraline and Risperidone. You're considering stopping Sertraline Tx @ Pt request

- In Bipolar long term antidepressants are often not ideal & their not taking them reliably anyway.

Then you learn during their last period of hypomania they had 3x suicide attempts

- This would make me far more cautious about stopping mood stabilizing medication.
- I would involve psychiatry & ensure close follow up care.
- Stopping Sertraline is not off the table, but it's a higher risk that must be carefully managed by someone more senior.

How Does This Impact Decision

Much less likely I'd deRx

Woman c Hyperthyroidism. You're suspecting Grave's Disease

- Grave's is high on DDx for Hyperthyroid

Then you learn that her TSH is NAD

- TSH should be extremely low in Grave's: TRAB force T_3 and T_4 release which inhibit TSH from pituitary.

Confidence in Dx

Much less likely

Woman c RA Presents c SOB. You're Think Interstitial Fibrosis

- RA commonly causes ILD, especially in long-standing disease.
- SOB in RA → ILD is a high-yield association.

Then you learn that she has been on MTX long term

- Methotrexate can cause methotrexate-induced pneumonitis, an acute or subacute inflammatory lung injury.
- This may be more common and more urgent than RA-ILD.
- It presents with dry cough, SOB, fever.

Her SOB is probably related to MTX use.

Confidence in original Dx

Much less likely

80F c Altered Mental State. You're thinking delirium

- This should be the default assumption.

You learn she start Oxybutnin for urinary incontinence

- Oxybutnin is a strong Anticholinergic drug - can cause confusion and agitation.
- In older people anti-Ach drugs are one of the top causes of delirium.

Confidence in original Dx

Confirmed

Man c dysphagia. You're thinking oesophageal strictures

- Progressive dysphagia → thinking structural cause.
- Stricture is common after chronic acid injury.

You discover he has had heartburn after meals for as long as he can remember

- Lifelong reflux → GORD-related peptic stricture becomes much more likely.
- Also raises suspicion for Barrett's oesophagus.

Confidence in original Dx

Much more likely

Man c dysphagia. You're thinking hiatal hernia

- Can cause reflux and mechanical problems leading to dysphagia.

Then you learn his Hb < 100

- Anaemia suggests chronic blood loss:
 - Caustic oesophagitis
 - Malignancy
 - Gastric Ulcer
- We should be concerned about malignancy.

Confidence in Dx

Much less likely

Woman c Nausea and Vomiting. You're thinking Hyperemesis gravidum

- Very common in the first trimester of pregnancy

Then you learn that her fundal height is 32 cm

- FH 32 cm → ~32/40 → Hyperemesis gravidum impossible.
- Need to consider late pregnancy disorders:
 - Pre-eclampsia
 - HELLP
 - Gastroenteritis
 - Obstruction

Confidence in Dx

Impossible

Woman c Flu-like Sx & Bilateral swollen joints. You're thinking Ross River Fever.

- Sx:
 - Fevers
 - Arthralgia
 - Myalgia
- Why are we thinking that first?

Then you learn that she is a childcare worker

- Childcare exposure to URTIs:
 - Vastly more common than arboviruses

Confidence in Dx

Very much less likely

Woman has 5 year Hx of migraines impacting her study. You're considering an MRI

- To rule out sinister sources.

Then you learn she has 0 neurological signs

- Stable, long standing, migraines without red flags do not require imaging



Would you image?

No.

Woman has 5 year Hx of migraines impacting her study. You're considering an MRI

- See above.

Then you learn the migraines have been getting progressively more severe and intense in the last 3/52

- Change in pattern and severity is a **RED FLAG**
- Even without focal signs, change in pattern suggests:
 - Tumour
 - Bleed
 - Raised ICP



Would you image?

Yes.

Elderly woman found on floor at home confused. You're thinking NOF fracture

- Common after a fall, especially with confusion

Then you learn she weight bear and walk

- NOFs make this impossible.



Confidence in Dx

Extremely unlikely

Woman presents c heavy periods. You're thinking of suggesting Mirena

- Near best Tx for heavy bleeding as acts locally.



Then you learn that her last pap smear came back with high grade abnormal histology

- Cervical pathology needs colposcopy/Mx first

Would you insert?

After Mx of suspected cervical pathology first.

Woman presents c heavy periods. You're thinking of suggesting OCP

- This is an effective strategy for Mx of heavy bleeding.

Then you learn she smokes 20 cigarettes per day

- Combined OCP + Smoking in >35 years is contraindicated due to DVT/Stoke risk.
- Heaving smoking risk = unacceptable risk.
- Consider mini-pill, Implanon, Mirena.



Would you prescribe?

No.

Woman c Abdominal pain. You're thinking Appendicitis requiring Appendectomy

- Central abdo pain that localises to the RLQ suggests appendicitis

Then you learn she has diffuse guarding and rigidity to palpation

- Suggests peritonitis
- Surgical emergency



How likely is Appendectomy now?

Extremely likely - urgent surgical RV.